



Referral, admission, and discharge Mental Health Hospital in the Home (MHHiTH) (Mental Health) Policy and Procedure

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Mission of service

To offer a contemporary, recovery-oriented alternative to inpatient admission for RKPG mental health patients by providing safe, high quality, evidence based and responsive mental health hospital level care in their home or usual place of residence.

Referral

MHHiTH accepts referrals from:

- Direct referral from the Emergency Department (ED) at Rockingham General Hospital (RGH) and Peel Health Campus (PHC). Fiona Stanley ED, Fremantle ED (this is if they present there and live in our catchment area)
- Mimidi Inpatient unit.
- Consultation Liaison Team (CL).
- Community Mental Health services (MHSs) (which encompasses multiple teams).
- Mother and Baby Unit, Fiona Stanley Hospital
- Referrals from other services which meet admission criteria see Appendix A.

Inclusion and exclusion criteria

A patient will only be able to come to the service if they meet the identified inclusion criteria for admission to MHHiTH. See Appendix B.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

Referral procedure

- Referrals will be received seven days per week 24 hours a day via email, this is only monitored in working hours 7 days a week.
- Referrals must be emailed to park.mhshithtriage@health.wa.gov.au
- Mental Health Assessment form SMHMR902 must be completed and sent through to the referral email address (see above) with a detailed discharge summary or detailed medical review, Risk Assessment and Management Plan (RAMP) and MHHiTH referral form.
- Referral to MHHiTH can be noted in the plan section on the SMHMR902 and on the subject area of the referral email.
- The Multidisciplinary team will discuss referrals with the MHHiTH CNM, CNS and Consultant during handover and the consultant will be responsible for accepting referrals to MHHiTH (Registrar may be the designated delegate).
- Referrer to liaise with MHHiTH to provide verbal handover and ascertain availability and also to discuss goals of care to establish treatment options.
- Referrals made outside of working hours will be discussed at the next handover meeting with the accepting Consultant or Registrar, for MHHiTH, will review.
- The MHHiTH Clinical Nurse manager (CNM) and Mental Health Clinical Nurse Manager Patient Flow (MHCNMPF) will be responsible for allocation of patient to MHHiTH bed via Enterprise Bed Management (EBM). See business rule and normal admission process as per:
 - * [Mental health bed Access, Capacity and Escalation Statewide Policy 2019](#)
 - * [SMHS Mental Health Patient Flow Management Plan](#)
- Also refer to:
 - * [HSS Enterprise Bed Management](#)
 - * [DoH Patient Activity Data](#).

Business rules for referrals and EBM

Referrals/EBM

- The MHHiTH CNM will discuss referrals with the Consultant or Registrar.
- The MHHiTH CNM will be responsible for liaising with the MHCNMPF in regards to referrals for MHHiTH.
- The MHHiTH CNM/senior nurse (weekend/public holiday) is responsible for monitoring referrals and ensuring that once accepted, EBM is updated and patients are placed in a MiTH bed on EBM within appropriate time frames.
- The MHHiTH CNM/Senior Nurse (weekend/public holiday) will be responsible for liaising with referring teams to ensure the appropriate documentation is complete.
- MHHiTH CNM will communicate with the MHCNMPF in regards to upcoming discharges in order to ensure patients prioritised for a MHHiTH bed are allocated beds in a timely manner.
- Weekend bed management will be the responsibility of the senior nurse for MHHiTH with input, if required from the Duty Nurse Manager (DNM) and Mimidi Shift Coordinator (if required).

EBM Community

Patients coming from community for admission:

- The Community Team Leader or case manager must put the patient on EBM for admission and send referral to the generic email (see above).
- If the community are unable to enter the patient on EBM they contact the MHCNMPF to enter

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

- patient onto EBM.
- The MHCNMPF will feedback to the community team of bed availability.

Suitability assessment

The CNM/Clinical Nurse Specialist (CNS) in conjunction with the Consultant or Registrar will review information provided by referrer to determine suitability in accordance with the referral criteria. The suitability assessment will encompass a review of the patient's presenting complaint, history, risk assessment as provided by the referring clinician in the Mental Health Assessment (SMHMR902) document and any other collateral history which may be deemed relevant. The purpose of the suitability assessment is to confirm that the clinical presentation can be safely managed by MHHiTH in a community setting; the patient is consenting to involvement and understands the commitment required.

Admission

Length of admission

- Patients will be admitted for up to 14 days.
- They will be admitted as per normal admission process for RKPG inpatient on
- EBM, web-based Patient Administration System (webPAS) and accompanying documentation completed in the psychiatric services online information system (PSOLIS) and iSOFT.
- There are some circumstances whereby patients are unsuitable for discharge from the MHHiTH at the end of the 14-day period such as:
 - * Continued requirement for inpatient management.
 - * Suitable follow-up not identified within the time (i.e. patient does not have adequate follow-up on discharge or follow-up is not within a timely period).
 - * The MHHiTH Consultant Psychiatrist is of the opinion that discharge is not yet appropriate.
- In these cases, the MHHiTH team will review the patient's needs for consideration the patient to be transferred to a hospital bed or discharged to another suitable setting, this will be planned 48 hours prior to the end of the 14 day admission.

Admission criteria

Admission criteria are outlined in Appendix A.

Consent

Patients will be required to sign a consent form see Consent to Treatment Form RGMR 54) agreeing to engage with the services. This will be completed by the clinician allocated to the patient.

Admission from Inpatient Unit

- If a patient is being referred and admitted from the inpatient unit there is an expectation the referring team will have commenced the discharge summary on Notification and Clinical Summary (NACs). The patient will also require a two week

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

supply of appropriate medication. The patient will be handed over to the MHHiTH team using iSoBAR handover.

- The allocated nurse will be required to collate relevant data for patient admission and call patient to go through all relevant documentation that can be completed prior to initial nursing assessment. The process will include, however, it not restricted to:
 - * Check PSOLIS for alerts and place in the consumer's integrated notes.
 - * Confirming the safety of visit as per [RKPG Community Home Visiting Policy](#).
 - * State-wide Standardised Clinical Documentation (SSCD) assessment if not already completed or requires review of existing SSCD after 24 hours of initially being done.

Initial nursing assessment

- This process happens in the patient's designated place where they will be residing during their treatment in MHHiTH.

Initial **nursing assessment** will occur within **24 hours** of patient admission to the MHHiTH service.

- The MHHiTH team will undertake a comprehensive initial assessment process at the first appointment at designated residence.
- Initial assessments for all new MHHiTH patients will be conducted by two MHHiTH staff members and may include support from the relevant community Case Manager family/carers or other supports.
- Patients transferring into MHHiTH from ED, Mimidi Park or Consultant Liaison Team will have initial assessment completed. However, the part of the pre-assessment will be to ensure this is in date and current for admission to MHHiTH.

Completion of documents

The MHHiTH allocated nurse will complete a:

- Treatment Support and Discharge plan
- Safety assessment to be documented in progress notes and recorded in iSOFT.
- iSoBAR handover to be completed iSOFT.
- Clinical observations to be taken and recorded in progress notes and on iSOFT
- Alcohol and other drugs (AoD)screen performed, and care planning initiated if
- recognition of need for support for use of AoD .
- Enter the consumer's details into PSOLIS and complete other admission and documentation requirements as per the Mental Health Inpatient Admission Documentation including, the [Department of Health \(DoH\) State-wide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)
- Expected date of discharge (EDD) will be entered into EBM.

Appointments

Consideration will be given to pre-existing appointments from community/medical services. The team will decide if the patient will still require these whilst admitted to the MHHiTH.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

The clerk will cancel these appointments as instructed by the allocated nurse/CNM/ Psychiatric Team.

Provision of information

The allocated nurse will

- Provide patient with advice regarding leave as per the:
 - * [RKPG Patient leave](#)
 - * [DoH Patient Activity Data Policy Information Compendium 2023](#)
- Provide an explanation of the following areas:
 - * Information regarding the proposed clinical care.
 - * Information on how to participate in decisions, treatment and care e.g. care planning and multidisciplinary team (MDT) meetings.
 - * Request to be seen by Psychiatrist.
 - * Roles, rights and responsibilities of consumer, guardian/ personal support person (PSP) and staff.
 - * Contact numbers for the MHHiTH, including afterhours access via online information (which can be printed to give to the patient).
 - * How to provide feedback and make complaints.
 - * Access to other services as required.

Patient/carer/guardian

- The allocated nurse is to:
 - * Ascertain if the patient consents to the carer/PSP being provided with information and being involved in their treatment or care.
 - * Ascertain if the patient would like to nominate an adult to advocate for their rights and interests (PSP as per Mental Health Act 2014).
 - * Provide information in a way that the patient and carer/PSP understands and document in the progress notes that all the above information has been explained to the consumer and carer/PSP if applicable.
- The allocated nurse will begin planning the patient journey including treatment, interventions and discharge planning with patient and carer/PSP input (if appropriate).

Medical assessment

The first **medical assessment** will occur within
48 hours of admission to MHHiTH

The initial medical appointment will involve:

- Assessment and diagnosis.
- Establishment of a biopsychosocial management plan.
- Medication review (at first medical appointment) and prescriptions as required.
- Mental Health Physical Examination Form (SMHMR903) - if this cannot be done at patient residence there is provision for it to be attended at Mimidi in the Older Adult Mental Health Treatment Room.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

- The admitting MO will ensure that:
 - * The risk assessment and patient safety plan is completed and documented; visual observations are noted including Risk Assessment and Management Plan (RAMP) (SMHMR905).
 - * Physical health examination is completed 48 working hours or as clinically indicated:
 - Patients transferred to MHHiTH from a ward will have had a physical health examination on admission. Any further physical health needs identified will be followed up by MHHiTH clinicians.
 - Patients, who have not previously been admitted to a ward or been assessed by ED, will have a full physical health examination conducted at the first medical appointment
 - Patients will be offered the option to have a chaperone and a choice whether to have the physical examination in their home environment.
- When indicated, MHHiTH clinicians may be required to monitor and record physical observations, such as the physical health monitoring of patients commenced on clozapine.
- Ensure all indicated tests and screens are initiated and results reviewed.
- Consent to treatment is confirmed and recorded if not already done by allocated nurse.
- If not already commenced the Registrar is to commence the Notifications and Clinical Summaries (NaCS) discharge summary so that these are available at the time and document expected date of discharge.

Continuation of admission

The MHHiTH team will:

- Complete clinical reviews of the patient face to face where possible and subsequently update the following:
 - * BOSSNET, PSOLIS, iSOFT (as required for handover).
 - * Inpatient Management Plan, including Patient Safety Plan.
 - * Estimated discharge date (EDD).
 - * RAMP.
 - * NaCS discharge summary – Registrars to update on an ongoing basis.
 - * National Outcome Case-mix collection (NOCC) as per standard protocols.
 - * Principal diagnosis and mental health legal status, as applicable.
- Complete MDT reviews that support patient treatment and discharge.
- Provide provision of specialised evidenced based mental health interventions in line with patient need. This may include further assessment, use of assessment tools, and or the use of therapy tools such as worksheets etc.
- Patient information will be handover over in iSOFT in an iSoBAR format as per:
 - * [RKPG Medical Clinical handover \(Acute\) Policy](#)
 - * [RKPG Clinical Handover-Nursing Midwifery \(Acute\) Guideline](#)
- Reviews may occur as part of a combined process using telehealth, where in the allocated nurse is with patient and communicating with other clinicians. This will provide a more accessible, comprehensive care.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

- Clinical teams must escalate to the Consultant Psychiatrist or Registrar or Duty MO (evening and afterhours) complex presentations and/or critical incidents.
- At any point during the admission the clinician may request a test for alcohol and other drugs this will be done as per [SMHS Alcohol and prohibited substances](#).

Discharge

- Discharge planning must commence on admission.
- Clear expectations of potential length of treatment must be provided on admission.
- All relevant discharge NOCC measures will be completed by the allocated nurse.

Discharge planning

- Discharge decisions should be planned, communicated, and facilitated as early as possible, ideally 48 hours prior to discharge, 7 days a week.
- The clinical team is to:
 - * Commence discharge planning upon the consumer's admission to MHHiTH and inform the patient and of the expected ongoing care including Expected date of discharge (EDD).
 - * The allocated nurse will ensure that all discharge information, liaison and arrangements have been addressed preferred 48 hours prior to discharge from the service.
 - * Involve patients in decisions about their care planning and also patient's carer as appropriate, taking into consideration maturity, consent, confidentiality, cultural needs and privacy as well as the nature and quality of carer's capacity.
 - * Consideration to be given to patient's risk history and use this information to inform the discharge plan when there is a history of poor engagement and non-attendance.
 - * Refer the patient's to support services (e.g. CMHS, General Practitioner (GP), non-government organisations) as soon as possible and as appropriate.
 - * Facilitate review of the patient by a member of the MHHiTH Medical Team/Consultant prior to discharge.
 - * If required, a planned discharge can occur on review by the On-call Psychiatrist afterhours, consideration to be given to telehealth in this instance.
 - * If discharge to a Community MHS is warranted and the patient is not already an active patient, discharge planning will include early contact with the local Community MHS Triage Officer to determine Case Manager allocation.
 - * Referral to other agencies will be completed as necessary prior to discharge.
- The care plan will be reviewed the day of discharge and will include final discharge care plan including:
 - * Discharge date.
 - * Destination (GP/CMHS/Private Practitioner).
 - * Care Transfer Summary.
 - * Medication List.
 - * Finalised discharge medication and script.
 - * Patient and carer survey provided and collected.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

- * 7 day follow up discussed.
- * All other relevant paperwork associated with the care plan finalised.
- Collaborative Action Plan, complete the discharge checklist as per the prior to discharge, including:
 - * Complete discharge medications (maximum of 2 weeks' worth discharge medication) or prescriptions. Patient counselling by visiting clinician in regards to any new medications, consider teleconference with pharmacist if required.
 - * Medical certificates (as required) prior to the EDD, ensuring consumers have appropriate discharge instructions/information.
- Re-referrals will be considered for re-entry into MHHiTH based on clinical need by way of an assessment of the consumer's current psychosocial context and presenting issues against the admission criteria.
- Upon discharge, patients, and family/carers/referrers as appropriate, are to be provided with information on re-entry to the service.

Transfer of care

- Care Transfer Summary (SMHMR916) completed.
- The MHHiTH team will liaise with relevant discharge services to ensure a seamless transfer of care. Where the patient is being discharged back to the care of a Community MHS, a discharge handover meeting should be arranged (where possible), in the days leading up to discharge to facilitate continuity of care.
- Handover can be done via phone call/videoconference via MS teams as per VC policy.
- If discharged to a GP or other service provider, a MHHiTH team member may attend with the patient for an appointment prior to discharge or link with the service via telehealth.
- MHHiTH patient requires admission to the Mimidi inpatient ward the MHHiTH episode of care will cease and the patient will be transferred, and care handed over to the care of the Mimidi team.

Community Treatment Orders

Clinicians must refer to the MHA 2014 for specific instruction regarding the discharge and transfer of consumers admitted under the MHA 2014.

Administration clerk role in admission/discharge

- Admitted into (webPAS) by the ward clerk or via hospital switchboard after hours.
- Update and maintain medical record whilst in MHHiTH service.
- Complete Patient Registration Form and process.
- Maintain admission documentation.
- Discharge patient as instructed by treating clinician.
- The patient is to be discharged administratively from PSOLIS, webPAS
- Patient and carer feedback is processed digitally via teams.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

Feedback

Feedback on the MHHiTH service is to be obtained by requesting the patient, carer, referrer and other relevant stakeholders to complete a satisfaction questionnaire either in hard copy or digitally.

Compliance monitoring

Compliance will be monitored as part of the:

- Clinical Incident Management (CIM) system process.
- Monitor unplanned readmissions.
- Compliments/Complaints Management System.
- Monthly reporting of KPIs and operational monitors to relevant Mental Health Divisional Clinical, Quality and Safety Committee including.
Referral time to MHHiTH to review by MHHiTH.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

Links

[DoH Acceptable Use of Information and Communications Technology Policy](#)

[DOH Chief Psychiatrist's Standards for Clinical Care](#)

[DoH Information and Communications Technology Governance Policy](#)

[DoH Information Security Policy](#)

[Department of Health \(DoH\) State-wide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)

[DoH Patient Confidentiality Factsheet](#)

[DoH Patient Activity Data](#)

[DoH Medication Review Policy](#)

[DoH Mental health bed access, capacity and Escalation State-wide policy](#)

[HSS Enterprise Bed Management](#)

[Mental Health Act 2014](#)

[RKPG Clinical Handover-Nursing Midwifery \(Acute\) Guideline](#)

[RKPG Community Home Visiting Policy.](#)

[RKPG Communication and Documentation Mental Health HiTH](#)

[RKPG Evening and After Hours MH HiTH](#)

[RKPG Medical Clinical handover \(Acute\) Policy](#)

[RKPG Patient leave](#)

[RKPG Physical Health Care: Community Mental Health Consumers \(Mental Health\) Policy and Procedure](#)

[RKPG Telehealth: Community Mental Health \(Mental Health\) Guideline](#)

[SMHS Alcohol and prohibited substances](#)

National Standards



RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

Appendix A: Admission Criteria

Criteria for admitted care to MHHiTH : Patient must qualify for a voluntary inpatient acute bed.	
- The primary clinical purpose or treatment goal is improvement in the symptoms and/or psychosocial, environmental and physical functioning related to a patient's mental disorder. - The patient must meet at least one of the following qualifications: * The patient requires expert clinical management and facilities that are only available in an inpatient ward or MHHiTH. * The patient requires daily assessment of their medication needs. * There is a requirement or social circumstances necessitating admission such as: - o Adult at risk (for example, inadequate level of social support to safely leave the hospital). - o The patient requires daily observation, risk assessment and plan to minimize self-harm risk.	
Mental health MHHiTH requirements	
- Resides in RKPG catchment area. - Home environment suitable for mental health care to be provided in the home. - Accommodation stable for next 14 days. - Person is suitable for a voluntary admission (will accept people on a CTO). - Aged 18-64.	
Clinical needs	
- Person has diagnosed mental health condition or acute mental health symptoms and is experiencing an acute episode of mental health symptoms/deterioration. - Severity of symptoms has worsened, and warrant admitted care setting OR - Person requires intensity of support not available in a community setting. - Patient would benefit from daily MHHiTH visits. - Physical health needs are stable or managed in the community. - AOD use is at a level that it will not impact or inhibit clinical care to a point that MHHiTH would not be beneficial to the patient.	
Risk	
- Risk of violence, aggression or self-harm can be managed in a community setting. - Home environment is safe for community visiting during daytime and evenings.	
Consent and participation	
Patient consents to participate with MHHiTH treatment on a daily basis and likely to engage with daily visits.	

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			

Appendix B: Inclusion and Exclusion Criteria for MHHiTH

Inclusion Criteria	Exclusion Criteria
- Resides in PaRK Catchment area - Home environment suitable for mental health care to be provided in and safe for community visiting both during daytime and evenings - Accommodation stable for next 14 days. - Person is suitable for a voluntary admission (will accept people on a CTO under shared care arrangement) - Aged 18-64 - Patient consents to participate with MHHiTH treatment on a daily basis and is likely to engage with daily visits - The primary treatment goal is improvement in the symptoms and/or psychosocial, environmental, and physical functioning related to a patient's mental disorder where: - The severity of symptoms has worsened and warrants an admitted care setting <i>and/or</i> - Person requires intensity of support not available in a community mental health setting - The patient must meet at least one of the following criteria: - The patient requires expert clinical management and facilities that are only: - Available in an inpatient ward or MHHiTH - The patient requires at least daily assessment of their medical care/medication needs. - It is anticipated that the mental health needs can be adequately met within a 14-day admission - Patient would benefit from daily MHHiTH visits	- Likely to require a psychiatric admission extending beyond 14 days. We take patients on clozapine titration for 4 weeks - With significant historical and current aggression risk and/or behavioural disturbance - Unwilling to engage - Patients residing in home environments that are unsafe or unsuitable for MHHiTH care to be provided - Who are likely to experience acute deterioration (physiological and/or mental state) once admitted to MHHiTH. This is determined by the referrer and assessed by the MHHiTH clinician on receipt of referral to MHHiTH. - Presenting with primary drug intoxication or withdrawal. - Presenting with complex and/or unstable medical problems, requiring admission to a general hospital bed. - Under the MHA 2014 - Aged under 18 years - Patients with significant unstable medical co-morbidities which may affect their mental state or the delivery of care - Patients being referred from private inpatient units (WA Health Admission Policy rules do not permit admission from a private hospital) - Posing a risk of transmissible diseases, for example, COVID 19, gastroenteritis, tuberculosis (TB) and influenza-like illness. We still provide service wearing PPE or we do phone calls instead of home visits - Note: The referring clinician may discuss the suitability of a consumer under the MHA 2014 with the MHHiTH Consultant, or delegate (i.e. Psychiatric Registrar) on a case-by-case basis

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Apr 2021	Revisions	NA	National Standards 1, 2, 5, 6
This version	May 2024	Revision Due	May 2027	
Responsibility of	Co-Directors M Health	Endorsed by	DONM/DCS	
Title	Referral, Admission and Discharge: Mental Health Hospital in the Home (Mental Health) Policy and Procedure			